Temozolomide with short-course radiotherapy, then adjuvant treatment

NCCP 00461 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking the treatment

Take the exact temozolomide capsule strengths and number on your pharmacy label, once daily when prescribed, on an empty stomach. Swallow whole with water; do not open, chew or crush. Ask the pharmacist to write the fasting interval and anti-sickness timing. Different phase doses are not interchangeable.

STEP 2

Missed or vomited medicine

If you vomit after temozolomide, do not take another dose that day. For a missed dose, use your pharmacist's written instructions or call before taking it late; do not double a dose. Never replace a dose held for toxicity.

STEP 3

Medicine and supplement review

Bring an up-to-date list of all medicines, including steroids, seizure medicines, anticoagulants, non-prescription treatments, vitamins and herbs. Ask before starting something new. Keep a record of all dose changes.

STEP 4

Storage and handling

Keep capsules in their original labelled packs, away from children. Do not open, crush or chew. If swallowing or medicine organisation is difficult, contact the team before the next dose and agree safe help. Return unused chemotherapy to the pharmacy.

STEP 5

Pregnancy, breastfeeding and fertility

Treatment can harm a pregnancy and may affect fertility. Discuss fertility preservation, contraception for patients and partners, and the required period after the final dose before starting. Do not breastfeed during treatment. Ask the team for written, medicine-specific advice.

STEP 6

Appointments and transport

Before starting: full blood count, kidney and liver tests, glucose and hepatitis B screening. Blood counts weekly through weeks 1–7, including four weeks after the last radiotherapy. Kidney and liver tests one and four weeks after radiotherapy. Adjuvant Day 1 blood count, kidney and liver tests each 28-day cycle; the four-week post-RT tests can form cycle 1 assessment. EMA additionally advises Day 22 blood count and after-cycle liver tests. MRI timing is individual.

STEP 7

Help with the daily plan

Ask a trusted person to help check the calendar if memory, vision, fatigue or hand strength makes treatment difficult. Confirm who is responsible for checking doses, collecting medicine, transport and calling for help. Keep consent and personal preferences central.

Sources: NCCP 00461 v3 · current Irish regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.